



Site	Service/Department/Unit	Disciplines
Fiona Stanley Hospital and Fremantle Hospital	Hospital Wide	Medical, Nursing, Pharmacy, Allied Health

Management of Agitation in Older Adults with Dementia or Delirium

This guide refers to patients with **PERSISTENT** behaviours such as:

- ▶ Restlessness, fidgeting
- ▶ Talking to self
- ▶ Shouting, swearing, arguing, insulting staff
- ▶ Pulling at lines and tubes
- ▶ Pacing
- ▶ Trying to leave the ward or hospital
- ▶ Continually trying to get out of bed
- ▶ Threatening or trying to hit, kick, bite or scratch

Behaviour like this is usually due to **DELIRIUM** or **DEMENTIA**

- ▶ Delirium may indicate a serious underlying medical or medication issue
- ▶ Agitated patients with dementia and/or delirium can injure themselves or others

This guideline is **NOT** for managing patients with:

- ▶ Simple insomnia
- ▶ Behaviour due to another psychiatric disorder such as mania or psychosis
- ▶ Simple threats to leave hospital against medical advice or conflict about a specific issue

STEP 1: Review diagnosis and address new problems first

- D** dehydration
E electrolyte imbalance
L level of pain
I infection/sepsis/inflammation
R respiratory failure (hypoxia/hypercapnia)
I impaction of faeces
U urine retention
M medication toxicity esp. anticholinergics (e.g. tricyclic antidepressants, oxybutynin), digoxin, sedatives/opioids, corticosteroids, anticonvulsants, antibiotics
- Consider alcohol or benzodiazepine withdrawal**

1

If withdrawal is suspected

- Do not use antipsychotics
- Initiate the Alcohol Withdrawal Scale (AWS) and refer to the *Alcohol and other Drugs: management of addiction, toxicity and withdrawal policy*

STEP 2: Assess risk and document plan

Consider potential for violence, pulling out tubes, absconding, interference with other patients, self and staff

2

If high risk

- Move to a single room
- Provide a companion
- Discuss with senior staff
- Consider medication

STEP 3: Non-pharmacological interventions: FIRST LINE management

See **CHAMPIONS**

- Avoid confrontation and physical restraints – walk away, defer interventions or offer food and drink to diffuse tension
- Single room where possible
- Call family to assist, ask them to stay
- Calmly engage, redirect and supervise the patient
- Maintain low level sensory stimulation
- Consider a companion
- Refer to the Sunflower Tool and other resources to support care

3

Pharmacological Management

Antipsychotic medications should be used with caution because of their association with strokes and cardiac events

- ▶ Aim to use one drug
- ▶ Avoid benzodiazepines, they can worsen delirium and cause falls and aspiration
- ▶ Review antipsychotic medications daily
- ▶ Keep doses to a minimum, higher doses are unlikely to help

If the patient has Parkinsonism or Lewy Body Dementia

Quetiapine 12.5mg to 25mg (tablet) 4 hourly PRN; maximum of 100mg in 24 hours.

If no response after 4 hours, move to Step 5 (olanzapine)

STEP 4: These are **calming** without much sedation

Haloperidol

250-500 micrograms (tablet, liquid, IM)
4 hourly PRN; maximum of 2mg in 24 hours

Risperidone

250-500 micrograms (tablet, liquid)
4 hourly PRN; maximum of 2mg in 24 hours

or

4

If the response is inadequate, escalate to senior staff

With senior review, move to Step 5

STEP 5: These are more **sedating**

Quetiapine

12.5mg – 25mg (tablet)
4 hourly PRN; maximum of 100mg in 24 hours

Olanzapine

2.5mg (tablet or oro-dispersible wafer)
4 hourly PRN; maximum of 7.5 mg in 24 hours

or

5

See also “Cognitive screening of the older adult” policy Appendix 5 (location: FSFHG Policy page)

Bibliography: Royal Perth Bentley Group, Clinical Practice Standard for Delirium Management; Australian Commission on Safety and Quality in Health Care, Delirium Clinical Care Standard

Higher doses than above may only be prescribed by a Registrar or Consultant